INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Temozolomide with radiotherapy, then adjuvant temozolomide

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
See the active treatment phase

Treatment stages
3
Explained in the treatment reference

Regimen source
NCCP 00334
Version 4

Take temozolomide daily throughout six weeks of radiotherapy, including weekends as prescribed. After a four-week break, take temozolomide on Days 1–5 of each 28-day cycle for up to six adjuvant cycles.

Treatment stages

01 **Temozolomide with six weeks of radiotherapy**
Daily capsule phase

02 **Four-week break before adjuvant treatment**
Recovery and review

03 **Up to six adjuvant 28-day cycles**
Five capsule days per cycle

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Use your written emergency plan even on days without chemotherapy. Fever, shivering or suddenly feeling unwell needs urgent advice. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. Follow any existing seizure rescue plan. Promptly report worsening headache, repeated vomiting, cough, breathlessness, unusual bruising, yellow skin or eyes, or inability to drink. A held chemotherapy dose must not be restarted without instructions.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

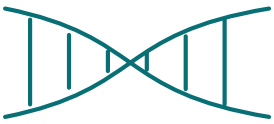
How your treatment works

This treatment combines daily temozolomide with radiotherapy, followed by a break and further temozolomide cycles, to help control newly diagnosed glioblastoma. Your team discusses the expected benefit and how it fits after surgery or biopsy.

ORAL CHEMOTHERAPY CAPSULE

Temozolomide

Temodal / generic



Damages deoxyribonucleic acid (the cell's genetic material) (DNA) in tumour cells

Temozolomide crosses into the brain and creates chemical changes in DNA. If the damage cannot be repaired, a cell may stop dividing or die. Healthy blood-forming cells can also be affected, which helps explain blood-count changes, infection and bleeding risks. Nausea prevention and liver monitoring are important too.

Capsule swallowed → DNA damaged → Cell growth disrupted

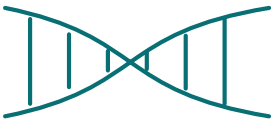
What to watch for

Report fever, bleeding, persistent vomiting, new breathing symptoms or yellow skin/eyes. Keep blood and liver tests even after capsule days finish.

TREATMENT FOCUSED ON THE BRAIN TUMOUR AREA

Radiotherapy

External-beam treatment planned by radiation oncology



Damages DNA in the treated area

Radiotherapy directs a planned dose of radiation to the tumour area. Radiation damages cellular DNA; planning aims to limit exposure of nearby healthy brain. The dose and treated area affect both benefit and side effects.

Individual planning scan → Radiation to planned area → Tumour-cell DNA damaged

What to watch for

Report new or worsening headache, vomiting, weakness, speech or memory change. The radiotherapy team gives a separate skin-care and symptom plan.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment and its purpose

Use the regimen and phase agreed with your team.

What each part does

Temozolomide produces chemical damage in DNA. Cells unable to repair that damage may stop dividing or die. Blood-forming cells can also be affected, which explains the need for blood counts and infection precautions. Radiotherapy damages DNA in the planned treatment area. Its team explains the separate dose, skin care and effects on the brain.

Schedule and planned duration

Take temozolomide daily throughout six weeks of radiotherapy, including weekends as prescribed. After a four-week break, take temozolomide on Days 1–5 of each 28-day cycle for up to six adjuvant cycles.

Evidence and uncertainty

Adding concurrent and adjuvant temozolomide improved survival in the trial population. This is evidence for the combined treatment strategy; it does not establish a separate benefit for every individual component or an individual survival forecast. The trial population and older pathology classification do not replace review of the current integrated diagnosis, fitness, O6-methylguanine-DNA methyltransferase (MGMT) result and patient priorities.

Understanding your particular glioma

Your diagnosis includes the tumour’s cells, molecular findings and behaviour, not just the word brain cancer.

The name and location

A glioma is a tumour arising in the brain or spinal cord. Astrocytoma, oligodendroglioma and glioblastoma are different types. Cancer that travels to the brain from another organ is a brain metastasis and follows a different pathway.

Grade is not the same as stage

Glioma reports combine the tumour type, grade and molecular findings. Grade is not a measure of spread around the body: a grade 4 brain tumour does not mean body-wide metastases. Ask your specialist to explain older terms in your records.

isocitrate dehydrogenase (IDH) and 1p/19q

Changes in the IDH genes and loss of chromosome regions 1p and 19q help classify certain gliomas. The pattern of results matters. These are usually tumour-sample tests; they do not by themselves show that a change runs in the family.

MGMT promoter methylation

The MGMT gene makes a protein that repairs a particular type of DNA damage. Methylation in its control region can reduce that repair and inform discussion of temozolomide benefit. This is a tumour-tissue result, not a blood marker or a personal survival percentage.

A useful diagnosis record

Keep the exact pathology diagnosis, sample date, grade and available molecular results together. Label a result as pending or not tested when appropriate. Ask whether further pathology review would affect the plan.

Your overall treatment plan and choices

Treatment should account for the tumour and the life you want to lead.

Understand the sequence

Place biopsy or surgery, radiotherapy, medicines and follow-up on one plan. Concurrent means treatments overlap; adjuvant means treatment follows the main treatment. A new phase can require different doses, tests and support medicines.

Agree what treatment is trying to achieve

Ask what treatment can reasonably achieve for your tumour and which effects matter most to you. Discuss benefits, burdens, alternatives and the reasons for a pause or stop.

Put evidence in context

A study result applies to a particular group, diagnosis and schedule. Older studies may use different tumour names. Ask which evidence fits your plan; an average cannot forecast your response or lifespan.

Discuss options available to you

Ask about suitable trials, another specialist opinion and other treatment or symptom-care options. Your multidisciplinary team explains what is appropriate and available for you.

Make room for your priorities

Bring up thinking, independence, family time, work and the side effects you most want to avoid. Ask for time, written notes or a supporter if that helps you weigh the choices.

Stupp EORTC/NCIC trial: radiotherapy with concurrent and adjuvant temozolomide

A randomised study compared radiotherapy alone with radiotherapy plus concurrent and adjuvant temozolomide in newly diagnosed glioblastoma.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Radiotherapy phase → recovery → adjuvant cycles

Treatment stages

01 **Temozolomide with six weeks of radiotherapy**
Daily capsule phase

02 **Four-week break before adjuvant treatment**
Recovery and review

03 **Up to six adjuvant 28-day cycles**
Five capsule days per cycle

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

DAILY CAPSULE PHASE

Temozolomide with six weeks of radiotherapy

Days 1–42 **Including weekends as prescribed**

Daily capsules and scheduled radiotherapy run together. Confirm the exact final capsule day and separate Pneumocystis jirovecii pneumonia (PJP)-prevention plan.

RECOVERY AND REVIEW

Four-week break before adjuvant treatment

No routine temozolomide **Keep planned blood tests**

Radiotherapy has ended, but safety checks and prescribed support medicines continue. The team checks recovery before another prescription.

FIVE CAPSULE DAYS PER CYCLE

Up to six adjuvant 28-day cycles

D1–5 capsules **D6–28 no temozolomide**

The first cycle has its own prescription; the team may change the dose only after reviewing tolerance and blood counts. Six cycles are the current NCCP plan.

The team confirms each phase and prescription. Do not use the capsule dose or test schedule from a different phase.

BASELINE → TREATMENT → FOLLOW-UP

Safety tests, symptoms and response review

The team books and reviews each test. Mandatory medicine-safety checks are distinct from individually timed magnetic resonance imaging (MRI) and tumour-tissue tests. There is no universal blood tumour marker for these treatment cycles.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00334 v4 · current regimen · EMA Temodal product information and patient leaflet

THE WHOLE TREATMENT JOURNEY

Temozolomide with radiotherapy, then adjuvant temozolomide: treatment and checks

Adults with newly diagnosed glioblastoma under NCCP 00334.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits / recovery	Next-cycle / response review	After a pause or stop
Blood count / kidneys / liver NCCP 00334	full blood count (FBC), renal/liver profile, glucose, hepatitis B surface antigen (HBsAg) and antibody to hepatitis B core antigen (anti-HBc)	FBC weekly during six-week radiotherapy (RT); kidneys/ liver before week 4	Continue arranged checks and recovery review before adjuvant treatment	Day 1 FBC/ renal/liver function tests (LFT) each cycle; Day 22 FBC under EMA guidance	LFT after cycles; follow delayed toxicity after the last dose
Neurology / medicines / function NCCP 00334	Record neurological symptoms, daily function and current medicines	Review seizures, headache, cognition, mood, walking and any steroid plan	Report changes early; do not wait for the next MRI	Review treatment tolerance, medication support and patient goals	Agree follow-up and rehabilitation or symptom support
MRI / diagnosis NCCP 00334	Confirm integrated diagnosis, molecular results and baseline imaging	MRI timing according to the neuro-oncology plan	Keep the booking and result-review contact in your passport	Interpret scan with symptoms, treatment effects and clinical review	Agree the next imaging and clinical review

What each check is for

1 **Temozolomide with six weeks of radiotherapy**

Daily capsule phase

This treatment combines daily temozolomide with radiotherapy, followed by a break and further temozolomide cycles, to help control newly diagnosed glioblastoma. Your team discusses the expected benefit and how it fits after surgery or biopsy.

Ask / act: Confirm the current phase, prescription, blood tests and support medicines.

2 **Four-week break before adjuvant treatment**

Recovery and review

This treatment combines daily temozolomide with radiotherapy, followed by a break and further temozolomide cycles, to help control newly diagnosed glioblastoma. Your team discusses the expected benefit and how it fits after surgery or biopsy.

Ask / act: Confirm the current phase, prescription, blood tests and support medicines.

3 **Up to six adjuvant 28-day cycles**

Five capsule days per cycle

This treatment combines daily temozolomide with radiotherapy, followed by a break and further temozolomide cycles, to help control newly diagnosed glioblastoma. Your team discusses the expected benefit and how it fits after surgery or biopsy.

Ask / act: Confirm the current phase, prescription, blood tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking the treatment

Take the exact temozolomide capsule strengths and number on your pharmacy label, once daily when prescribed, on an empty stomach. Swallow whole with water; do not open, chew or crush. Ask the pharmacist to write the fasting interval and anti-sickness timing. Different phase doses are not interchangeable.

STEP 2

Missed or vomited medicine

If you vomit after temozolomide, do not take another dose that day. For a missed dose, use your pharmacist's written instructions or call before taking it late; do not double a dose. Never replace a dose held for toxicity.

STEP 3

Medicine and supplement review

Bring an up-to-date list of all medicines, including steroids, seizure medicines, anticoagulants, non-prescription treatments, vitamins and herbs. Ask before starting something new. Keep a record of all dose changes.

STEP 4

Storage and handling

Keep capsules in their original labelled packs, away from children. Do not open, crush or chew. If swallowing or medicine organisation is difficult, contact the team before the next dose and agree safe help. Return unused chemotherapy to the pharmacy.

STEP 5

Pregnancy, breastfeeding and fertility

Treatment can harm a pregnancy and may affect fertility. Discuss fertility preservation, contraception for patients and partners, and the required period after the final dose before starting. Do not breastfeed during treatment. Ask the team for written, medicine-specific advice.

STEP 6

Appointments and transport

Before starting: full blood count, kidney and liver tests, glucose and hepatitis B screening. During concurrent treatment: blood count weekly; kidney and liver tests before week 4. Adjuvant treatment: blood count, kidney and liver tests on Day 1 of every 28-day cycle. EMA additionally advises a Day 22 blood count with follow-up until recovery and liver tests after each cycle. MRI timing is individual.

STEP 7

Help with the daily plan

Agree who can help with medicines, travel and appointments if memory, sight, fatigue or hand strength makes them difficult. Record your preferences and consent for sharing information.

Taking treatment safely

The exact prescription and calendar matter.

Missed or vomited medicine

Medicine and supplement review

Tests, urgent symptoms and follow-up

Keep planned tests and know how to get help.

Support medicines

Temozolomide has moderate-to-high oral emetic potential. Take the prescribed anti-sickness plan before chemotherapy and use rescue medicine as instructed. Ask what to do if nausea prevents taking medicines or fluids. PJP-prevention medicine is required during temozolomide with radiotherapy in this NCCP regimen. Your team supplies the prescription and decides how long to continue, including if lymphocyte counts remain low. Do not stop it just because radiotherapy has finished. Anti-sickness medicines and other medicines can worsen constipation. Ask for your bowel plan, and call for persistent vomiting, severe constipation or difficulty drinking. Do not double a dose after vomiting. If prescribed, keep steroids and seizure medicines on separate written schedules. Chemotherapy rest days do not tell you when to stop them. Ask the pharmacist to check interactions and explain any dose changes.

A delay is a treatment decision

Blood counts, organ tests or symptoms may mean a dose is held, reduced or stopped. This is a safety decision, not a reason to take extra capsules later. Ask which medicines change, which support medicines continue and when the next assessment is due.

After the final dose

Agree follow-up blood tests, symptom review, MRI timing and who to contact. Continue to report new problems; effects can outlast chemotherapy. Keep your medicine list and written plan current.

Tests and scans: what each one tells you

Safety blood tests, tumour tissue tests and MRI answer different questions.

Safety blood tests

A full blood count measures red cells, white cells and platelets. Kidney and liver tests assess other aspects of treatment safety. Follow the blood-test schedule even when you feel well or the scan is reassuring.

MRI and neurological review

MRI gives detailed pictures of the brain. The team also checks symptoms, movement, speech, thinking and daily function. Before scanning, tell staff about implants, metal, kidney problems or previous contrast reactions, and ask for help if the scan worries you.

Changes on a scan need interpretation

Treatment-related changes can resemble tumour growth on MRI; this is called pseudoprogression. The specialist interprets the scan with symptoms and may need another scan or further assessment. Report worsening symptoms promptly rather than assuming a scan change is harmless.

An individual follow-up timetable

Record the next MRI and the contact who will explain it. Your diagnosis, treatment phase and previous results determine the interval. Other scans or blood markers are ordered for a specific reason, not automatically before every cycle.

Keep results with decisions

Mark tests as booked, done and reviewed so the next action is clear. Contact the named service about a missing result. Do not change chemotherapy or steroids in response to a report without instructions.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Temozolomide once daily	 **Days 1–5** Use the current phase prescription, fasting and with the prescribed nausea plan.
No temozolomide capsules	 **Days 6–28** Recovery, symptoms and blood/liver tests; restart only when authorised.
Day 22 blood count	 **Day 22** EMA recommends a count around Day 22 and follow-up until recovery.
Safety throughout the cycle	 **Days 1–28** Fever, bleeding, neurological or breathing symptoms need prompt advice.

Dose changes belong to the current phase: Daily chemoradiotherapy and five-day cycles use different doses. Check the label and phase with the pharmacist whenever a new supply is issued.

Effects can be delayed: Blood counts and liver effects can change after capsule days finish or after the last cycle. Keep the follow-up tests and emergency plan.

No replacement after vomiting: If you vomit after a temozolomide dose, do not take a second dose that day. For a missed dose, follow the pharmacist's written instructions or contact the team; never double up.

Why these bands stay broad: This 28-day view is the ADJUVANT phase only. The earlier daily chemoradiotherapy phase and four-week break are shown separately above. Symptoms can occur on any day. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00334 v4 · current regimen ↗

There are no guaranteed "safe days". Fever, shivering or suddenly feeling unwell needs urgent advice, including on treatment rest days. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. Follow the emergency instructions in any established seizure rescue plan.

Between-treatment notes

Day 1–5: Five daily capsule doses

Check the prescribed capsule strengths and number each day. Take fasting with the anti-sickness plan. Do not use leftover capsules to change the dose or replace a vomited dose.

Day 6–21: No temozolomide; recovery continues

Rest days allow recovery but blood counts can still fall. Continue other medicines only as individually prescribed. Contact the team for fever, sudden illness, bruising or persistent vomiting.

Day 22–28: Blood count and next-cycle decision

Have the Day 22 blood count and any follow-up blood tests arranged. Before the next cycle, the team reviews counts, kidney/liver tests and symptoms. Do not start the next cycle just because the calendar reaches Day 1.

Brain symptoms, medicines and when to call

Symptoms may reflect the tumour, swelling, treatment, another medicine or a separate illness.

Know your usual symptoms

Before treatment, note your usual walking, strength, speech, sight, memory, sleep and seizure pattern. Record new changes and their timing. A trusted person can help if you want them involved.

Steroids need a separate written plan

Steroids may reduce swelling around a brain tumour. If needed, ask for a written dose, review date and reduction plan; do not stop suddenly or adjust the dose yourself. Report infection symptoms, marked thirst, weakness or troublesome sleep or mood changes.



Seizure medicines and rescue instructions

Take prescribed seizure medicine on its own schedule. Ask what to do after a missed or vomited dose and how to manage a breakthrough seizure. Do not switch or stop it yourself. Anyone giving a rescue medicine needs the personal instructions and training.

Safety at home and outside

Discuss driving, work, bathing, swimming, heights and machinery with your team, taking account of symptoms and seizures. Individual medical and licensing advice is needed. Occupational therapy can help make daily routines safer.

Living well, rehabilitation and support

Practical and symptom support can begin alongside anticancer treatment.

Thinking, communication and daily tasks

Tell the team about changes in memory, language, attention, sight or planning. Rehabilitation can involve physiotherapy, occupational therapy, speech and language therapy or psychology. Ask for practical help with routines and medicines.

Food, nausea, bowels and energy

Use your prescribed nausea and bowel plan and report problems early. Ask for dietitian help if intake or weight falls. Adapt movement to your balance and strength, and ask the team to assess fatigue that disrupts daily life.

Emotional support and carers

Tell the team about changes in mood, behaviour or coping. Patients and carers may each need psychological or practical support. Agree who can receive information and ask for help before caring becomes overwhelming.

Work, transport and finances

Plan travel, time away from work, caring duties and help with appointments. A nurse or social worker can identify practical support; agree who can help with tablets if needed.

Supportive care and future wishes

Supportive and palliative care can help manage symptoms while treatment continues. When you are ready, discuss future care preferences and whom you want involved. Ask your team to help document these wishes.

Information that works on paper

Keep the latest paper schedule, medicine list, results plan and emergency contacts together. The treatment card helps others identify your regimen. It supports, but does not replace, the prescription and emergency instructions.

Questions worth asking

What is my exact diagnosis, grade and relevant molecular result?

Ask for the integrated pathology wording and an explanation of IDH, 1p/19q and MGMT where relevant. The diagnosis and treatment history guide how this regimen applies to you.

What is my PJP-prevention plan, including after radiotherapy?

The NCCP concurrent treatment requires PJP prevention. Ask which medicine, when to take it, monitoring and when the team will decide it can stop. Low lymphocyte counts may mean continuing longer.

Who helps with seizures, steroids, memory or daily tasks?

Ask for a named contact, written seizure and steroid plans if prescribed, and rehabilitation or practical support. Agree who can help you organise treatment.

When will we review the benefit and my priorities?

Ask for the next clinical and MRI review, how treatment effects will be distinguished from growth, and what would lead to continuing, changing or stopping treatment.

Try explaining the plan back: Can you show the days for each medicine, the current phase, your next blood tests and whom to call for fever, vomiting or new neurological symptoms?

Temozolomide + radiotherapy (RT) Adults with newly diagnosed glioblastoma under NCCP 00334. Use the plan matched to your diagnosis and phase; do not borrow another glioma schedule.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Concurrent daily temozolomide with radiotherapy

Active medicines: Temozolomide

Emetogenic risk: Moderate to high (oral)

Medicine	Dose / route	When
Oral 5-HT3 antagonist - select ONE	Ondansetron 8-16 mg total daily OR granisetron 1-2 mg total daily; prescriber selects and records agent/ dose	Daily during the concurrent temozolomide course, including non-radiotherapy days; confirm the written stop date NCCP V6 Table 5, p8. Start at least 30 minutes before oral systemic anticancer therapy (SACT) (p4). Confirm stop date, delayed-emesis cover and rescue plan; review QT risk, liver function, constipation and interactions.
Pneumocystis jirovecii pneumonia (PJP) prevention	Complete the local prophylactic agent, dose and schedule	Required throughout concurrent temozolomide/radiotherapy; review continuation after RT Continue when clinically indicated for lymphopenia/steroid exposure. Do not stop automatically at the end of RT; confirm allergy, renal function, interactions and pharmacy plan.

Additional phase-specific support

- Temozolomide has moderate-to-high oral emetic potential. Take the prescribed anti-sickness plan before chemotherapy and use rescue medicine as instructed. Ask what to do if nausea prevents taking medicines or fluids.
- PJP-prevention medicine is required during temozolomide with radiotherapy in this NCCP regimen. Your team supplies the prescription and decides how long to continue, including if lymphocyte counts remain low. Do not stop it just because radiotherapy has finished.
- Anti-sickness medicines and other medicines can worsen constipation. Ask for your bowel plan, and call for persistent vomiting, severe constipation or difficulty drinking. Do not double a dose after vomiting.
- If prescribed, keep steroids and seizure medicines on separate written schedules. Chemotherapy rest days do not tell you when to stop them. Ask the pharmacist to check interactions and explain any dose changes.
- These are prescription choices for clinician/pharmacist completion, not an issued prescription. General NCCP multiday guidance includes cover after the last SACT day; record the individual duration. Review pre-existing steroid and seizure treatment rather than duplicating it.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Adjuvant temozolomide Days 1–5 every 28 days

Active medicines: Temozolomide

Emetogenic risk: Moderate to high (oral)

Medicine	Dose / route	When
Oral 5-HT3 antagonist - select ONE	Ondansetron 8-16 mg total daily OR granisetron 1-2 mg total daily; prescriber selects and records agent/ dose	Before temozolomide on Days 1-5; confirm the prescribed stop date and delayed-emesis cover NCCP V6 Table 5, p8. Start at least 30 minutes before oral systemic anticancer therapy (SACT) (p4). Confirm stop date, delayed-emesis cover and rescue plan; review QT risk, liver function, constipation and interactions.

Additional phase-specific support

- Temozolomide has moderate-to-high oral emetic potential. Take the prescribed anti-sickness plan before chemotherapy and use rescue medicine as instructed. Ask what to do if nausea prevents taking medicines or fluids.
- Continue or review Pneumocystis jirovecii pneumonia (PJP) prevention according to lymphocyte recovery, steroid exposure and the specialist plan. Starting five-day adjuvant cycles does not itself authorise stopping prevention prescribed during radiotherapy.
- Anti-sickness medicines and other medicines can worsen constipation. Ask for your bowel plan, and call for persistent vomiting, severe constipation or difficulty drinking. Do not double a dose after vomiting.
- If prescribed, keep steroids and seizure medicines on separate written schedules. Chemotherapy rest days do not tell you when to stop them. Ask the pharmacist to check interactions and explain any dose changes.
- These are prescription choices for clinician/pharmacist completion, not an issued prescription. General NCCP multiday guidance includes cover after the last SACT day; record the individual duration. Review pre-existing steroid and seizure treatment rather than duplicating it.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Brain / energy

Tumour / treatment

Fatigue or headache may occur. New seizures, sudden weakness, speech difficulty or loss of consciousness need emergency help.

3 Blood / infection

Temozolomide

Counts can fall after capsule days finish. Fever, sudden illness, bruising or bleeding needs urgent advice; keep all blood tests.

5 Liver

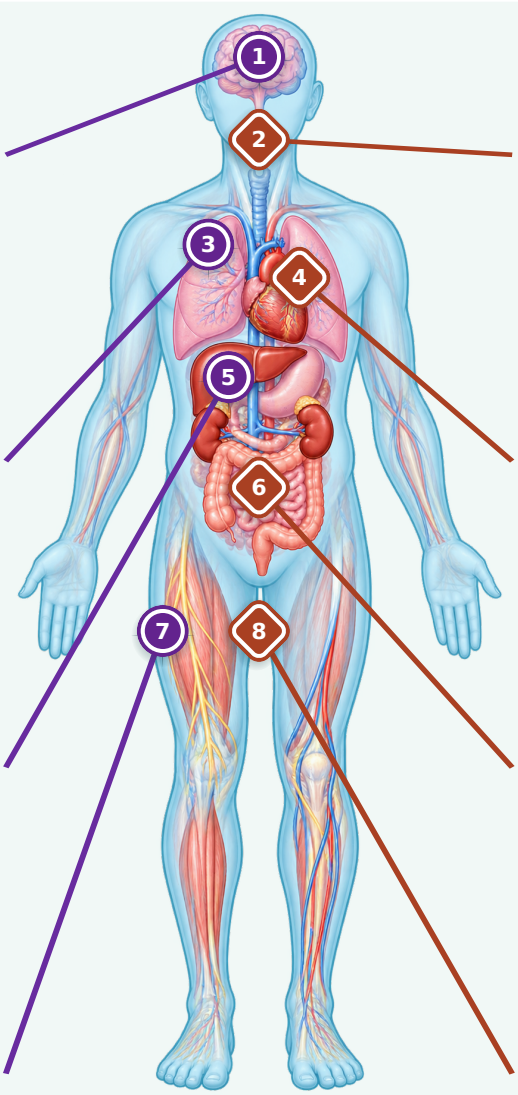
Temozolomide

Liver injury may appear during or after treatment. Attend liver tests and report yellow skin or eyes, dark urine or unusual itching.

7 Skin / scalp

Radiotherapy / temozolomide

Follow scalp care during RT; report new rash, blisters or peeling skin. Hair loss and skin irritation depend on the treatment received.



2 Mouth / eating

Temozolomide

Report mouth soreness, ulcers or reduced intake. Keep gentle mouth care and ask for help before eating or drinking becomes difficult.

4 Lungs / PJP

Temozolomide

New cough, fever or breathlessness needs prompt review. Follow the prescribed PJP-prevention plan during chemoradiotherapy.

6 Nausea / bowels

Temozolomide / support medicines

Take anti-sickness medicine as prescribed. Call for repeated vomiting, inability to drink, severe constipation or abdominal pain.

8 Fertility / pregnancy

Temozolomide

Discuss contraception and fertility before starting. Do not breastfeed during treatment. Ask for the written medicine-specific precautions.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Use your oncology unit’s written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions. Contact the team promptly for a worsening headache, repeated vomiting, new cough or breathlessness, unusual bruising, yellow skin/eyes or symptoms that prevent drinking. Do not restart held chemotherapy without instructions.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital’s temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00334 v4 · current regimen**
   https://healthservice.hse.ie/documents/6960/334_V4_Temozolomide_radiotherapy (RT)_and_Adjuvant_Therapy.pdf
2. **EMA Temodal product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/temodal-epar-product-information_en.pdf
3. **Stupp EORTC/NCIC trial: radiotherapy with concurrent and adjuvant temozolomide**
   https://pubmed.ncbi.nlm.nih.gov/15758009/
4. **NCI: adult central nervous system tumours · diagnosis and grade education**
   https://www.cancer.gov/types/brain/patient/adult-brain-treatment-pdq
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